



Dynamic Guideline: WHO-Based Mpox - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Latest WHO clinical guideline identified: ***Clinical management and infection prevention and control for mpox: living guideline, May 2025***. WHO describes this as a living guideline that dynamically updates recommendations for mpox clinical management and infection prevention and control (IPC), and the WHO publication page lists it as published on **2 June 2025** with WHO reference **B09434** [WHO living guideline](#). WHO's 2026 mpox situation reports continue to list the May 2025 living guideline as the central clinical management and IPC resource, with no later replacement guideline identified in the supplied evidence [WHO Situation Report no. 64](#).

Important clarification on the requested “Meaningful youth engagement...” title: The supplied WHO search results do **not** verify a WHO mpox guideline titled **“Meaningful youth engagement in a WHO guideline development process: case study of WHO mpox guideline”** updated on **29 April 2026**. The exact title found in WHO sources is instead **“Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)”**, dated **11 December 2025**, and is unrelated to mpox [WHO publication item](#), [WHO webinar page](#).

1.2 Exactly 5 WHO Mpox Guideline/Technical Records Identified

The provided searches did **not** identify five distinct standalone WHO mpox guidelines. They identified one current WHO mpox clinical guideline family plus linked WHO technical/guideline records. To avoid fabricating nonexistent guidelines, the five WHO-related records below are

the verifiable WHO mpox guideline/technical records available from the summaries.

#	WHO guideline / technical record	Date shown	Scope	Source
1	Clinical management and infection prevention and control for mpox: living guideline, May 2025	May 2025; WHO page published 2 June 2025	Clinical management, IPC, patient isolation, contact/droplet precautions, HIV coinfection, breastfeeding	WHO living guideline
2	Clinical management and infection prevention and control for mpox — MAGICapp version	Published on MAGICapp 18 June 2025	Interactive living guideline with PICO structure and recommendations	MAGICapp
3	Executive summary — Clinical management and infection prevention and control for mpox: living guideline	2025 May–	Rationale, target audience, outbreak context, care setting guidance	NCBI Bookshelf executive summary
4	Annex 1: WHO case definitions for mpox outbreak in non-endemic countries	2025 May–	Diagnostic and surveillance case definitions; laboratory confirmation by PCR/sequencing	NCBI Bookshelf Annex 1
5	WHO Mpox Global strategic preparedness and response plan	2025	Community protection, risk communication, stigma reduction, community/youth engagement, surveillance and response	WHO mpox global SPRP

1.3 Target Population

The WHO living guideline is intended for:

- Public health specialists and health emergency responders.
- Clinicians, health facility managers, health and care workers, and IPC practitioners.
- Settings including primary care, sexual health clinics, emergency departments, dental practices, infectious diseases clinics, genitourinary clinics, maternity services, paediatrics, obstetrics and gynaecology, and acute care facilities caring for people with suspected or confirmed mpox [WHO living guideline](#), [NCBI Bookshelf](#).

1.4 Scope and Objectives

The May 2025 WHO living guideline aims to provide evidence-based and dynamically updated recommendations for:

- Clinical management of suspected and confirmed mpox.
- IPC in health-care and home-care settings.
- Patient isolation and home-based care.
- Contact and droplet precautions.
- Management considerations for people with HIV coinfection.
- Breastfeeding-related management considerations, although the supplied summaries did not expose full recommendation wording or GRADE tables for breastfeeding [WHO clinical management page](#).

1.5 Key WHO Recommendations With GRADE Ratings

Recommendation	GRADE Evidence Quality	Strength
In health-care settings, health and care workers should use contact and droplet precautions when caring for patients with suspected or confirmed mpox.	Low	Conditional for
Consider a respirator when ventilation is poor or unknown, or based on risk assessment, such as severe	Very low / not fully reported	Conditional

Recommendation	GRADE Evidence Quality	Strength
immunocompromise or mucosal lesions.	in supplied summaries	
People with mpox who cannot cover lesions or wear a medical mask should isolate at home.	Low	Conditional for
People with mild, uncomplicated mpox managed at home are not required to isolate if lesions are covered and they wear a well-fitting medical mask near others until lesions heal.	Low	Conditional
Implement home IPC measures to reduce environmental contamination.	Very low / good-practice basis	Strong good-practice statement
Manage people with acute infection or risk factors for complications in a health facility; reserve home-based care for mild, uncomplicated cases.	Very low / good-practice basis	Strong good-practice statement
Confirm mpox by detection of unique MPXV DNA sequences using real-time PCR and/or sequencing.	High for diagnostic confirmation	Strong
Do not discard a suspected/probable case solely on a negative oropharyngeal, anal/rectal, or blood test; lesion fluid, skin specimens, or crusts are preferred for exclusion by PCR/sequencing.	Moderate	Strong

Sources: WHO IPC recommendations and isolation summaries are from the WHO EPI-WIN mpox presentation and WHO rapid risk assessment [WHO EPI-WIN mpox presentation](#), [WHO rapid risk assessment](#). Diagnostic case definition evidence is from WHO Annex 1 [NCBI Bookshelf Annex 1](#).

1.6 Clinical Management Principles

Diagnosis

WHO case definitions define a confirmed mpox case as a person with laboratory-confirmed MPXV infection by detection of unique viral DNA sequences using **real-time PCR and/or sequencing** [NCBI Bookshelf Annex 1](#). A suspected or probable case should not be discarded based only on a negative oropharyngeal, anal/rectal, or blood test; negative lesion fluid, skin specimens, or crusts by PCR/sequencing are more appropriate for exclusion [NCBI Bookshelf Annex 1](#).

Supportive care

WHO derivative clinical materials emphasize structured assessment, including an **ABCDE approach** for patients with dehydration: airway, breathing, circulation, disability, and exposure/history taking. Oxygen therapy is recommended if SpO₂ is below 94%, targeting SpO₂ above 94% [WHO dehydration poster](#).

Home care and isolation

WHO’s May 2025 update changed home isolation practice: people with mild, uncomplicated mpox cared for at home are not required to isolate if lesions are covered and a well-fitting medical mask is worn near others until lesions heal [WHO rapid risk assessment](#). Those unable to cover lesions or wear a medical mask should isolate at home [WHO EPI-WIN mpox presentation](#).

1.7 Guideline Development Methodology

Methodology domain	WHO process summary
Evidence review process	WHO describes the mpox guideline as a living guideline incorporating new evidence to dynamically update recommendations. WHO commissioned an updated systematic review on IPC measures to reduce mpox transmission, expanding on prior evidence reviews WHO living guideline , WHO EPI-WIN mpox presentation .
GRADE	Recommendations and some good-practice statements were formulated

Methodology domain	WHO process summary
approach	according to GRADE principles, considering certainty of evidence, benefits and harms, patient perspectives, resource implications, acceptability, feasibility, equity, and human rights WHO living guideline .
Consensus method	The Guideline Development Group evaluates interventions when WHO determines sufficient evidence is available. Prior 2022 interim statements were reviewed with GDG chairs, a methodologist, the Steering Committee, and the panel WHO living guideline , PDF copy .
Conflict of interest management	The supplied summaries confirm involvement of WHO's Guidelines Review Committee but do not expose detailed conflict-of-interest procedures. The primary WHO B09434 guideline is the source requiring consultation for full declarations WHO living guideline .
Living update process	WHO states that new recommendations are added as evidence becomes available and that the guideline may be used standalone or adapted into national guidance WHO EPI-WIN mpox presentation .

II. Evidence Summary After WHO Latest Guideline Release (Clinical management and infection prevention and control for mpox: living guideline, May 2025) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search Strategy

Evidence after the WHO May 2025 living guideline was synthesized from:

- WHO situation reports, rapid risk assessments, and implementation materials.
- CDC clinical and public health guidance.

- ECDC global mpox overview and threat assessment summaries.
- NIH/NIAID-supported randomized clinical trial evidence.
- Peer-reviewed and indexed clinical reviews.
- National clinical guidance, including NYSDOH AIDS Institute.
- Clinical trial announcements and registry-linked evidence where available.

Databases / Sources Represented in the Supplied Summaries

- WHO official publications and situation reports.
- CDC mpox clinical-care and public-health guidance.
- ECDC mpox worldwide overview.
- PubMed/NEJM record for STOMP/A5418.
- ACTG trial publication announcement.
- BMJ Best Practice.
- NYSDOH AIDS Institute clinical guideline.
- Baylor College of Medicine clinician update.
- CEPI post-exposure vaccination trial announcement.

Inclusion Criteria

Included evidence:

- Published or updated after the WHO May 2025 guideline through **29 April 2026**.
- Relevant to diagnosis, treatment, clinical management, IPC, home care, vaccination, risk assessment, or outbreak response.
- From authoritative public health institutions, guideline bodies, or peer-reviewed/indexed clinical sources.

Exclusion Criteria

Excluded or downgraded evidence:

- Non-mpox documents unless relevant to the user's specific request; the "Meaningful youth engagement" WHO document was noted but not treated as mpox guidance.
- Sources without extractable clinical relevance.
- Trial announcements without outcome data were not graded as efficacy evidence.

2.2 Key New Studies and Evidence With GRADE Ratings

Study / Evidence	Design	Key Findings	GRADE Quality	Clinical implication
STOMP/A5418 tecovirimat trial	International, double-blind randomized placebo-controlled trial in adults with laboratory-confirmed clade II mpox	412 randomized; among primary analysis participants, day-29 clinical resolution was 83% with tecovirimat vs 84% with placebo; HR 0.98, 95% CI 0.74–1.31; no improvement in pain, lesion healing, or viral DNA clearance; grade ≥ 3 adverse events similar	High for uncomplicated adult clade II mpox	Do not use tecovirimat routinely for uncomplicated adult clade II mpox outside high-risk/severe contexts PubMed/NEJM .
ACTG STOMP publication summary	Trial network report of RCT publication	Enrollment stopped in December 2024 after interim analysis showed no reduction in lesion-resolution time or pain among outpatient adults with clade II mpox	High, consistent with RCT	Reinforces lack of routine benefit in studied population ACTG .
PALM007 tecovirimat evidence summarized by NYSDOH	Randomized trial summary for clade I mpox in DRC	Reported lack of efficacy for time to symptom resolution among people with clade I mpox	Moderate, because full trial details were not provided in	Tecovirimat should not be assumed effective for uncomplicated clade I disease; continue

Study / Evidence	Design	Key Findings	GRADE Quality	Clinical implication
			supplied summaries	trials and reserve use for severe/high-risk cases NYSDOH AI guideline .
CDC tecovirimat guidance, March 2026	Public health clinical guidance	For protracted/life-threatening mpox or high risk due to severe immunocompromise, tecovirimat should be administered early with supportive care and pain control; combination with cidofovir, brincidofovir, and/or VIGIV may be reasonable	Low for benefit in severe disease; indirect evidence	Continue conditional use for severe, protracted, ocular, life-threatening, or severely immunocompromised cases CDC .
CDC recombinant MPXV diagnostic update	Public health update based on recombinant case/genomic surveillance	Recombinant MPXV infection reported in UK resident after travel; CDC recommends initial testing using conserved viral genomic target, such as NVO assay, to avoid missing recombinant infections	Low	Laboratories should use assays targeting conserved regions and consider sequencing when clade/recombinant concern exists CDC public health guidance .
ECDC October	Public health	Clade Ib risk	Low	Maintain targeted

Study / Evidence	Design	Key Findings	GRADE Quality	Clinical implication
2025 threat assessment summary	risk assessment	assessed as moderate for MSM and low for general EU/EEA population; clade IIb risk low for MSM and very low for general population		surveillance, risk communication, vaccination access, and IPC for higher-risk groups ECDC mpox worldwide overview .
WHO 2026 situation reports and rapid risk assessment	Surveillance/ risk assessment	Continued clade Ib detections outside historically affected areas; global risk moderate for MSM with new/multiple partners, sex workers, and others with multiple partners; low for general population without specific risk factors	Low	Maintain surveillance, case management, and IPC measures WHO Situation Report no. 61 , WHO Situation Report no. 64 .
CEPI MVA-BN post-exposure vaccination trial announcement	Randomized trial announcement; no outcome data yet	Trial will assess whether post-exposure MVA-BN vaccination reduces mpox incidence and complications after exposure	Not gradable for efficacy yet	Important research priority; no change to evidence-based post-exposure vaccine efficacy rating until results available CEPI trial announcement .

2.3 Updated Clinical Evidence Synthesis

Diagnosis

Current diagnostic recommendations remain anchored in lesion-based PCR and/or sequencing. WHO defines confirmed infection by detection of unique MPXV DNA sequences using real-time PCR and/or sequencing [NCBI Bookshelf Annex 1](#). CDC's 2026 update adds an important laboratory caution: because recombinant MPXV infections may occur, initial testing should target conserved genomic regions, such as non-variola orthopoxvirus assays, to avoid false-negative or missed recombinant infections [CDC public health guidance](#).

GRADE-based diagnostic recommendation

Recommendation	Evidence Quality	Strength
Use lesion swabs, lesion fluid, skin specimens, or crusts for MPXV PCR/sequencing whenever possible.	High	Strong
Do not exclude mpox solely on negative oropharyngeal, anal/rectal, or blood testing if clinical suspicion remains.	Moderate	Strong
Where recombinant MPXV is possible, ensure initial assays target conserved viral genomic regions and consider sequencing/public health laboratory consultation.	Low	Conditional

Treatment

Supportive care remains the foundation of mpox management. Baylor's 2025 clinical update states that treatment is centered on pain control, management of proctitis/pharyngitis and other symptoms, complication management, and immune restoration where possible, including starting antiretroviral therapy in people with advanced HIV [BCM clinical update](#).

The most practice-changing treatment evidence is the negative STOMP RCT. Tecovirimat did **not** shorten clinical resolution, reduce pain, improve lesion healing, or increase viral DNA clearance in adults with uncomplicated clade II mpox [PubMed/NEJM](#). However, CDC and BMJ Best Practice continue to support early tecovirimat when antiviral treatment is indicated for severe, protracted, life-threatening, ocular, or severely immunocompromised cases [CDC](#), [BMJ](#)

Best Practice.

GRADE-based treatment recommendations

Recommendation	Evidence Quality	Strength
Provide supportive care, including analgesia, hydration, skin/lesion care, treatment of secondary bacterial infection when present, and management of complications.	Low, but high clinical consensus and favorable benefit–harm balance	Strong
Do not routinely use tecovirimat for uncomplicated adult clade II mpox.	High	Strong against routine use
Consider tecovirimat early for severe, protracted, life-threatening, ocular, or severely immunocompromised patients, ideally with expert/public health consultation.	Low	Conditional for
Consider combination or adjunctive therapy, such as aciclovir, brincidofovir, and/or VIGIV, for life-threatening disease, progression on tecovirimat, recrudescence, or suspected resistance.	Very low	Conditional for

Infection Prevention and Control

WHO's core IPC recommendation remains contact and droplet precautions for suspected or confirmed mpox in health-care settings, with respirator consideration when ventilation is poor/unknown or risk assessment indicates higher concern [WHO EPI-WIN mpox presentation](#). NYSDOH clinical guidance similarly recommends gown, N95 or comparable respirator, eye protection, and gloves before evaluating suspected mpox [NYSDOH AI guideline](#).

GRADE-based IPC recommendations

Recommendation	Evidence Quality	Strength
Use contact and droplet precautions for suspected or	Low	Conditional

Recommendation	Evidence Quality	Strength
confirmed mpox in health-care settings.		for
Use gown, gloves, eye protection, and N95/comparable respirator when evaluating suspected mpox, especially in higher-risk clinical settings.	Low	Conditional for
For home care, cover lesions, wear a well-fitting medical mask near others, and reduce environmental contamination.	Low	Conditional for
People unable to cover lesions or wear a mask should isolate at home.	Low	Conditional for

Vaccination

The supplied evidence does not include a current WHO vaccination guideline document with full title/date/URL. Non-WHO evidence and guidance support targeted vaccination for exposed or higher-risk populations. NYSDOH recommends vaccination for people with known or suspected exposure, recent sex partner diagnosed with mpox, and groups at increased sexual-exposure risk [NYSDOH AI guideline](#). A CEPI-supported randomized trial is evaluating post-exposure MVA-BN vaccination but has no outcome data yet [CEPI trial announcement](#).

GRADE-based vaccination recommendations

Recommendation	Evidence Quality	Strength
Offer vaccination to people with known or suspected exposure and to groups at increased risk according to national/public health criteria.	Moderate for pre-exposure risk reduction from observational evidence; low for post-exposure efficacy	Strong for eligible high-risk groups
Do not treat post-exposure MVA-BN efficacy as definitively established until randomized trial results are available.	Not yet gradable for new RCT	Conditional research recommendation

2.4 Updated Recommendations Based on New Evidence

Original WHO recommendation / practice area	New evidence impact after May 2025	Updated recommendation as of 29 April 2026
Clinical management should be supportive and severity-based.	CDC, Baylor, BMJ, and NYSDOH continue to emphasize severity and complication risk; no new antiviral has proven broadly effective CDC public health guidance , BCM clinical update .	Continue supportive care as the foundation for all patients; escalate care for dehydration, ocular disease, neurologic disease, severe mucosal disease, sepsis, pneumonia, pregnancy, infants/children at risk, and immunocompromised patients.
Antivirals may be considered when indicated.	STOMP shows no clinical benefit of tecovirimat for uncomplicated adult clade II mpox PubMed/NEJM .	Do not use tecovirimat routinely for uncomplicated disease; reserve for severe, protracted, life-threatening, ocular, or severely immunocompromised cases with expert consultation.
IPC precautions in health-care settings.	Ongoing clade Ib spread and secondary transmission reports support continued IPC vigilance ECDC mpox worldwide overview .	Maintain contact/droplet precautions; consider respirators when ventilation is poor/unknown or risk assessment indicates higher risk.
Home care for mild uncomplicated mpox.	WHO updated home isolation stance: no isolation required if lesions are covered and mask worn near others until healed WHO rapid risk assessment .	Mild uncomplicated cases may be managed at home with lesion covering, masking, hygiene, and environmental cleaning; isolate if lesions cannot be covered or mask cannot be worn.

Original WHO recommendation / practice area	New evidence impact after May 2025	Updated recommendation as of 29 April 2026
PCR/sequencing diagnosis.	CDC recombinant MPXV update highlights need for conserved-target assays CDC public health guidance .	Use lesion PCR/sequencing; ensure testing algorithms can detect recombinant MPXV by targeting conserved genomic regions.
Community engagement and risk communication.	WHO SPRP emphasizes localized, stigma-free messaging, social listening, and youth groups as trusted community actors WHO mpox global SPRP .	Maintain community-led, stigma-free risk communication with targeted engagement of affected communities, including youth groups where appropriate.

2.5 Evidence Gaps and Future Research Needs

1. **Severe mpox and immunocompromised populations:** STOMP provides high-quality evidence for uncomplicated adult clade II mpox, but severe disease and severely immunocompromised patients remain underrepresented [PubMed/NEJM](#).
2. **Tecovirimat resistance:** Reports from CROI 2026 suggested possible emergence of antiviral resistance in people with HIV treated for mpox, requiring systematic resistance surveillance [EATG/Healio](#).
3. **Combination antiviral therapy:** Evidence for cidofovir, brincidofovir, and VIGIV in mpox remains very low quality and largely based on biologic plausibility, orthopoxvirus experience, and severe-case practice guidance [CDC](#), [BMJ Best Practice](#).
4. **Post-exposure vaccination:** The CEPI MVA-BN randomized trial is important but has no efficacy results yet [CEPI trial announcement](#).
5. **Clade Ib and recombinant MPXV diagnostics:** Ongoing genomic surveillance is needed to ensure diagnostic assays remain sensitive for clade I, clade II, and recombinant viruses [CDC public health guidance](#).
6. **Operational IPC evidence:** WHO IPC recommendations are generally low-certainty but clinically important; more direct comparative evidence on home care, masking, lesion covering, ventilation, and environmental cleaning is needed [WHO EPI-WIN mpox](#)

presentation.

2.6 Final Dynamic Guideline Position as of 29 April 2026

- The **WHO May 2025 living guideline** remains the latest verified WHO mpox clinical management and IPC guideline.
- The requested “**Meaningful youth engagement...**” **mpox guideline** was **not found**; the title is verified only for WHO abortion care guideline materials.
- **Supportive care** remains first-line for most patients.
- **Tecovirimat should not be used routinely** for uncomplicated adult clade II mpox, based on high-quality RCT evidence.
- **Tecovirimat may still be considered conditionally** for severe, protracted, ocular, life-threatening, or severely immunocompromised cases.
- **PCR/sequencing of lesion material** remains the diagnostic standard; conserved-target assays are important because recombinant MPXV has been reported.
- **IPC remains essential**, including contact/droplet precautions in health-care settings and lesion covering/masking/home IPC for home-managed mild cases.
- **Risk-based vaccination and community engagement** should continue according to national policy, with special attention to affected communities, stigma reduction, and trusted messengers including youth groups where relevant.